

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

September 16, 2022

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

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CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C
Chief, Investigations Unit
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FROM:

LCDR (b)(6),(b)(7)(C),(b)(7)(F), RN
Senior Investigator
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Benjamin GONZALEZ-Soto, (b)(6),(b)(7)(C)

Executive Summary:

On July 8, 2022, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Benjamin GONZALEZ-Soto, (b)(6),(b)(7)(C). During his detention period, ICE housed Mr. GONZALEZ-Soto, a 36-year-old Mexican male, at Florence Staging Facility (FSF) within Florence Service Processing Center (FSPC) in Florence, Arizona (AZ), from July 1 – 7, 2022, and Central Arizona Florence Correction Complex (CAFCC) in Florence, AZ, from July 7 until July 8, 2022, the date of his death. An autopsy ruled Mr. GONZALEZ-Soto's cause of death was spontaneous bacterial peritonitis (an infection of abdominal fluid from an unknown source).

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Prior to ICE custody, on June 29, 2022, the Phoenix Police Department arrested Mr. GONZALEZ-Soto for possession/use of drug paraphernalia and obstruction of highway/public thoroughfare. On that same date, ICE lodged an Immigration Detainer – Notice of Action, Form I-247, with the Maricopa County Sheriff's Office (MCSO). On June 30, 2022, MCSO transferred Mr. GONZALEZ-Soto to ICE Enforcement and Removal Operations (ERO) Phoenix, and ICE issued Mr. GONZALEZ-Soto a Notice to Appear (NTA), Form I-862, charging him with violating Section 212(a)(6)(A)(i) of the Immigration and Nationality Act (INA), as a noncitizen present in the United States without being admitted or paroled. On July 1, 2022, ERO Phoenix transferred Mr. GONZALEZ-Soto to FSPC in Florence, AZ, to await receipt of the prior removal documentation from the National Records Center.

On July 1, 2022, during his FSPC intake screening, Mr. GONZALEZ-Soto denied any medical history, drug, or alcohol use.

On July 2, 2022, during Mr. GONZALEZ-Soto's initial physical exam, he reported using fentanyl (a synthetic opioid) daily for the last year and experiencing body aches, nausea, vomiting, and mild anxiety for two to three days. Between July 2 and 7, 2022, FSPC's health staff treated Mr. GONZALEZ-Soto for opiate withdrawal.

On July 7, 2022, ICE ERO transferred Mr. GONZALEZ-Soto to CAFCC. During his CAFCC intake screening, Mr. GONZALEZ-Soto complained of body aches, nausea, abdominal pain, and agitation. A registered nurse (RN) contacted the on-call provider via telephone, who gave the RN a verbal order to admit Mr. GONZALEZ-Soto into medical observation.

On July 8, 2022, at approximately 1:23 a.m., Mr. GONZALEZ-Soto experienced cardiac arrest. CAFCC initiated cardiopulmonary resuscitation (CPR), which continued for approximately seven minutes until American Medical Response (AMR) emergency medical services (EMS) and the Florence Fire Department (FFD) arrived and assumed care. At 1:41 a.m., FFD paramedics contacted Mountain Vista Medical Center (MVMC), in Mesa, AZ, and a physician pronounced Mr. GONZALEZ-Soto deceased.

Mortality Finding:

As a result of Mr. GONZALEZ-Soto's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined Mr. GONZALEZ-Soto's medical care at FSPC and CAFCC was not provided within the safe limits of practice. Specifically, the MRC determined the following: 1) FSPC did not provided medical care in accordance with IHSC

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policy and clinical guidelines; 2) FSPC did not refer Mr. GONZALEZ-Soto to the emergency room (ER) timely; 3) FSPC custody staff did not communicate pertinent information to the medical department; 4) CAFCC did not refer Mr. GONZALEZ-Soto to the ER timely; and 5) CAFCC medical staff inconsistently reviewed written critiques of man-down drills.

The MRC convened on August 25, 2022, and it consisted of the following IHSC members: Dr. (b)(6),(b)(7)(C),(b)(7)(F) clinical director (CD), physician reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) western regional advanced practice provider (APP), APP reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) IHSC chief nurse, and Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) central regional nurse manager, nurse reviewers; and (b)(6),(b)(7)(C),(b)(7)(F) risk management supervisory nurse, risk management reviewer.

The following summary of events was based on: 1) Mr. GONZALEZ-Soto's FSPC and CAFCC medical records; 2) Mr. GONZALEZ-Soto's EMS report; 3) incident and notification reports; 4) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 5) Mr. GONZALEZ-Soto's FSPC and CAFCC detention file; 6) Mr. GONZALEZ-Soto's autopsy report; 7) video footage from July 7 – 8, 2022; 8) an on-site review and staff interviews conducted by fact finder Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) at FSPC and CAFCC between August 9 – 11, 2022; 9) applicable FSPC and CAFCC policies and procedures; and 10) applicable ICE Detention Standards.

Summary of Events

On July 1, 2022, a licensed practical nurse (LPN) completed Mr. GONZALEZ-Soto's intake screening using interpreter service in his native language (Spanish), documented no medical history, and normal vital signs (VS). Mr. GONZALEZ-Soto denied a history of drug and alcohol use. The LPN completed Mr. GONZALEZ-Soto's coronavirus disease 2019, which showed negative results.

On July 2, 2022, an APP completed Mr. GONZALEZ-Soto's physical examination. Mr. GONZALEZ-Soto reported using fentanyl or methamphetamine (a highly addictive and potent central nervous system stimulant) daily, for the past year (last use of fentanyl: June 28, 2022), experiencing body aches, nausea, vomiting, mild anxiety for two to three days, and a history of opiate withdrawal in April 2022. The APP documented normal VS, completed a clinical opiate withdrawal scale (COWS) assessment (scale used to measure the severity of opiate withdrawal symptoms), documented his score of six (scores below 12 indicate mild withdrawal; 13-24 indicates moderate withdrawal; 25-36 indicates moderately severe withdrawal; and scores above 36 indicate severe withdrawal), and ordered the following: clonidine (antihypertensive medication used to treat withdrawal symptoms) 0.1 milligrams (mg), promethazine (anti-nausea medication) 25 mg, and ibuprofen 600 mg, three times daily, as needed, for five days; and a COWS assessment three times daily.

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On July 3, 2022, ICE transferred Mr. GONZALEZ-Soto to the Phoenix Field Office. FSPC nursing staff completed Mr. GONZALEZ-Soto's three COWS assessments, approximately eight hours apart, and documented scores of seven, four, and six, respectively.

- At approximately 4:32 p.m., an APP completed Mr. GONZALEZ-Soto's emergent evaluation for his complaint of weakness, chills, nausea, and body aches. The APP documented normal assessment findings, normal VS, and because FSPC does not have a medical housing unit, ordered his placement in the FSF Special Housing (FSH) unit (an administrative segregation unit) to allow custody staff to monitor him more closely.

On July 4, 2022, nursing staff completed Mr. GONZALEZ-Soto's three COWS assessments, approximately eight hours apart, and documented scores of four, three, and seven, respectively.

- At approximately 11:00 a.m., an APP re-evaluated Mr. GONZALEZ-Soto, documented normal VS, a normal assessment, and scheduled an APP follow-up in three days. Mr. GONZALEZ-Soto denied abdominal pain but reported body aches, chills, and nausea.

On July 5, 2022 at approximately 4:07 a.m., ICE transferred Mr. GONZALEZ-Soto to the Phoenix Field Office, issued him an NTA/Warrant of Arrest, and returned him to FSPC for removal proceedings.

- At approximately 7:08 p.m., Mr. GONZALEZ-Soto returned to FSPC. Nursing staff completed Mr. GONZALEZ-Soto's COWS assessment and documented a score of five. FSPC's medical staff did not readmit Mr. GONZALEZ-Soto into the FSH unit due to its isolated location and limited visibility.

On July 6, 2022, nursing staff completed Mr. GONZALEZ-Soto's three COWS assessments, approximately eight hours apart, and documented scores of five, twelve, and six, respectively.

- At 9:50 a.m., the CD evaluated Mr. GONZALEZ-Soto for chest pain, documented normal VS, ordered an electrocardiogram (records electrical signal from the heart to check for different heart conditions) that showed normal results, and reordered his opiate withdrawal medications for seven days.
- At 12:51 p.m., the CD evaluated Mr. GONZALEZ-Soto for abdominal pain and vomiting, documented normal VS, a normal abdominal assessment, and ordered medication to treat his nausea and vomiting, and one liter of intravenous (IV) normal saline (IV fluids used to hydrate and replenish electrolytes).
- At 9:00 and 10:30 p.m., Mr. GONZALEZ-Soto continued to experience severe abdominal pain and vomiting. An RN documented his heart rate (HR) of 140 beats per

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minute [(bpm); normal range 60–100 bpm], blood pressure (BP) of 102/70 millimeters of mercury [(mmHg); normal range 90/60–120/80 mmHg], and notified the on-call APP, who instructed the RN to re-evaluate him the following morning.

On July 7, 2022 at 4:30 a.m., an RN completed Mr. GONZALEZ-Soto's COWS assessment and documented his score of 10. Mr. GONZALEZ-Soto reported vomiting throughout the night but improvement in his abdominal pain, which he rated an eight on a 10-point scale. The RN documented the following VS: HR 128 bpm, 130 bpm, and 96 bpm; BP 80/56 mmHg; and respirations (R) 23 and 22 breaths per minute (normal range 12–20 breaths per minute). The RN notified the on-call provider, who gave a verbal order for Mr. GONZALEZ-Soto to receive a higher-level evaluation with the CD upon his arrival on duty that morning; however, due to unforeseen circumstances, the CD did not evaluate Mr. GONZALEZ-Soto as ordered.

- At 7:51 a.m., an APP completed Mr. GONZALEZ-Soto's follow-up examination. Mr. GONZALEZ-Soto reported having nausea, vomiting, abdominal pain, body aches, insomnia (difficulty sleeping), and agitation. An LPN documented the following VS: HR 137 bpm, systolic BP 78 mmHg, R 28 breaths per minute. The APP documented a normal abdominal exam, except for diffuse tenderness, ordered two liters of IV normal saline, and scheduled an APP follow-up evaluation for July 8, 2022.
- Between 8:48 and 11:00 a.m., prior to inserting Mr. GONZALEZ-Soto's peripheral IV catheter, an RN documented the following VS: HR 137 bpm, BP 98/70 mmHg, and R 28 breaths per minute. While he received IV fluids, a BHP evaluated Mr. GONZALEZ-Soto for his substance abuse history. After administering Mr. GONZALEZ-Soto's IV fluids, the RN completed his COWS assessment, documented his score of three, and VS: HR 109 bpm, BP 116/94, and R 25 breaths per minute. Meanwhile, another RN completed Mr. GONZALEZ-Soto's medical transfer summary and provided a CAFCC RN with Mr. GONZALEZ-Soto's current status via phone and email. After Mr. GONZALEZ-Soto received the IV fluids, the APP documented improvement of his symptoms and instructed him to notify medical staff with any worsening symptoms prior to discharging Mr. GONZALEZ-Soto to his dorm.
- At approximately 4:00 p.m., custody staff called the medical clinic and notified an RN that Mr. GONZALEZ-Soto complained of abdominal pain. The RN informed the custody staff member that Mr. GONZALEZ-Soto would be called to the clinic later that day for a scheduled appointment.
- At approximately 4:43 p.m., prior to his medical appointment, ICE transferred Mr. GONZALEZ-Soto to CAFCC along with his medical transfer summary. Prior to his transfer, an FSPC RN recorded Mr. GONZALEZ-Soto's normal temperature and observed him entering the custody transport van without difficulty.

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On July 7, 2022 at 5:17 p.m., Mr. GONZALEZ-Soto arrived at CAFCC.

- At 7:18 p.m., a CAFCC correctional officer (CO) notified medical staff of Mr. GONZALEZ-Soto's complaints of withdrawal symptoms and observed him lying on the floor "in the fetal position." An RN evaluated Mr. GONZALEZ-Soto, completed his COWS assessment, and documented his score of eight along with the following VS: HR 117 bpm, BP 126/86 mmHg. Mr. GONZALEZ-Soto returned to the holding cell to await further processing.
- At 8:47 p.m., custody staff escorted Mr. GONZALEZ-Soto to the CAFCC medical waiting area to await medical intake screening.
- At 11:30 p.m., an RN completed Mr. GONZALEZ-Soto's intake screening, recorded his active medications, documented "sickly appearance," dilated pupils, COWS score of 12, and the following VS: HR 122 bpm, BP 134/107 mmHg, R 22 breaths per minute. During his intake screening, Mr. GONZALEZ-Soto asked the RN to allow him to use the restroom to urinate. Mr. GONZALEZ-Soto denied feeling nauseous or needing to vomit.

On July 8, 2022 at 12:03 a.m., an RN notified an APP of Mr. GONZALEZ-Soto's medical history, who gave a verbal order to admit him into medical observation. Mr. GONZALEZ-Soto waited in the medical waiting area while CAFCC prepared his room in medical observation.

- At approximately 12:46 a.m., other detainees notified the RN that Mr. GONZALEZ-Soto vomited a dark-brown fluid in the bathroom trash can, and the RN completed a visual inspection of the trash cans in the medical waiting area. The RN did not immediately notify the APP that Mr. GONZALEZ-Soto vomited or of its appearance.
- At 1:05 a.m., Mr. GONZALEZ-Soto fell when exiting the bathroom in the medical waiting area and struck his head on a nearby bench. Other detainees alerted the CO, who responded to the area with an RN. The RN evaluated Mr. GONZALEZ-Soto, noted a deep laceration above his right eye, his altered level of consciousness and unclear speech, and requested additional medical staff to respond.
- At 1:09 a.m., an RN requested the CO call a medical emergency via facility radio in response to Mr. GONZALEZ-Soto's decreasing level of consciousness. An RN observed Mr. GONZALEZ-Soto's shallow respirations and administered naloxone (medication used to treat suspected opioid overdose). CAFCC medical staff administered five doses of naloxone throughout their resuscitative efforts.
- At 1:14 a.m., CAFCC contacted emergency medical services via telephone. Medical staff attempted to obtain Mr. GONZALEZ-Soto's VS and blood sugar without success, noted

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dark colored fluid coming from Mr. GONZALEZ-Soto's mouth, and turned him on his side.

- At 1:23 a.m., Mr. GONZALEZ-Soto experienced cardiac arrest. CAFCC initiated CPR, attached an automated external defibrillator that advised "no shock," and continued CPR for approximately seven minutes until AMR and FFD assumed his care.
- At 1:41 a.m., FFD paramedics contacted MVMC, and a physician pronounced Mr. GONZALEZ-Soto deceased.

The final autopsy report revealed Mr. GONZALEZ-Soto's cause of death was spontaneous bacterial peritonitis and within other significant conditions, listed severe coronary artery atherosclerosis (narrowing of the blood vessels that supply blood to the heart). The coroner determined the manner of death as natural.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, PBNDS 2008, and the National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

FSPC and CAFCC Strengths

Strengths identified during this investigation were: 1) FSPC's medical staff routinely communicate with receiving facilities when transferring detainees who are being monitored for alcohol and/or opiate withdrawal; 2) FSPC's nursing staff consistently documented Mr. GONZALEZ-Soto's COWS assessments; 3) FSPC's medical staff clearly documented the information provided to CAFCC prior to Mr. GONZALEZ-Soto's transfer for continuity of care purposes; 4) CAFCC's medical staff routinely complete a brief screening immediately upon a new detainee's arrival; 5) CAFCC's medical and custody staff are trained on the use of naloxone and are empowered to administer it if opioid withdrawal is suspected; and 6) CAFCC completed an internal mortality review and shared the findings with all medical staff.

FSPC and CAFCC Weaknesses

1. Medical care.

- On July 2, 2022, FSPC nursing staff did not document an assessment to justify a referral to a higher-level provider.
- On July 2, 2022, during Mr. GONZALEZ-Soto's initial physical examination at FSPC, the APP did not order routine labs, in accordance with the clinical guidelines. In addition, providers did not consider differential diagnoses.

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- IHSC policy requires the CD or physician designee to provide oversight and supervision of all detainees with substance abuse issues, active intoxication, and medically supervised withdrawals. Mr. GONZALEZ-Soto's medical records do not clearly reflect this oversight from his date of arrival.
- FSPC medical staff did not initiate a medical hold for Mr. GONZALEZ-Soto, and ICE repeatedly transferred him to the Phoenix Field Office and to CAFCC during his opiate withdrawal monitoring.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section K. *Substance Dependence and Detoxification*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section N. *Medical/Psychiatric Alerts and Holds*.
- NCCHC: Standards for Health Services in Jails, 2018: J-F-04, *Medically supervised withdrawal and treatment*.
- IHSC Policy: IHSC Directive 03-13, 2021. *Detainees with substance use, intoxication, and withdrawal*.
- Federal Bureau of Prisons Clinical Guidance, 2020: *Medically Supervised Withdrawal for Inmates with Substance Use Disorders*.

2. Untimely access to emergency care.

- On July 6, 2022, nursing staff did not document the use of the urgent emergent clinical nurse guidelines during Mr. GONZALEZ-Soto's emergent visits, which would have prompted health staff to consider referring him to the ER.
- On July 6 and 7, 2022, Mr. GONZALEZ-Soto experienced signs and symptoms warranting referral to the ER; however, neither the physician nor the APP referred him to the ER timely.
- On July 7, 2022, FSPC custody staff notified medical staff of Mr. GONZALEZ-Soto's continued complaint of abdominal pain; however, medical staff did not evaluate him immediately, and ERO transferred him to CAFCC prior to his scheduled appointment.
- On July 7, 2022, during his CAFCC intake screening, Mr. GONZALEZ-Soto experienced signs and symptoms warranting immediate referral to the ER; however, CAFCC did not call EMS until almost two hours later.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.

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- IHSC Policy: Interim Reference Sheet for New Clinical Nursing Guidelines, Appendix B – Adult Urgent Emergent Clinical Nurse Guidelines, 2021. *Abdominal Pain*.
- PBNDS 2008: Part 4, Medical Care; section O. *Emergency Medical Services and First Aid*.
- CAFCC Policy: 13-34, *Medical Emergency Response*.

3. Communication.

- On July 7, 2022, FSPC custody staff observed Mr. GONZALEZ-Soto with a trash bag containing dark brown emesis, but there is no evidence to confirm whether they notified the medical staff for further evaluation/inspection of the contents.
- FSPC health staff sent CAFCC emails that contained inaccurate information regarding Mr. GONZALEZ-Soto's last fentanyl use, which resulted in confusion at CAFCC.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section Z. *Continuity of Care*.
- PBNDS 2011 with 2016 Revisions: Part 7.4, Detainee Transfers; section C. *Responsibilities of the Health Care Provider at the Sending Facility*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-07, *Nonemergency Health Care Requests and Services*.

4. Staff Training.

- Some CAFCC staff did not attend man down drills and could not confirm review of past man-down drill critiques.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2018: J-D-07, *Emergency Services and Response Plan*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, CAFCC's and FSPC's administrator and health authority, for review and any action(s) deemed appropriate.

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- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.